MX-CONNECT

INSTITUTIONAL PRESENTATION NOTE
NOVEMBER 2026

MX-CONNECT

From mutual solidarity to Universal Health Coverage

A digital platform that modernises health mutuals, guides people towards Universal Health Coverage and strengthens the complementarity between community and public health protection mechanisms.



Ministry of Public Health

Ministry of Labour and Social Security

Ministry of Youth Affairs and Civic Education

ARAILD

Presented by
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“

THE ESSENTIALS IN THREE SENTENCES

01 **The health mutual protects its members according to its own benefits.**

02 **UHC provides coverage within the national scheme.**

03 **MX-CONNECT facilitates coordination and complementarity between the two.**

Joining a health mutual is not enrolment in Universal Health Coverage (UHC), and UHC enrolment does not make anyone a member of a mutual. MX-CONNECT preserves this distinction. It equips health mutuals and turns them into community partners for information, guidance and support towards the national scheme, without ever taking its place.

CONTENTS

Purpose of this note and positioning P. 03

I. Situation analysis P. 04 – 08

II. The MX-CONNECT platform P. 09 – 10

III. Health mutuals and UHC: two distinct, complementary mechanisms P. 11 – 16

IV. Contributions to other public policies P. 17

V. How it works, screen by screen P. 18 – 21

VI. Requests, roadmap and indicators P. 22 – 23

Sources · Contact P. 24 – 25

PURPOSE OF THIS NOTE

A public-interest tool serving policies already under way

WHAT THIS NOTE IS NOT

A commercial offer. MX-CONNECT sells no service to the ministries and never holds funds.

WHAT IT PROPOSES

An operational instrument, already online, that connects health mutuals, health facilities, organisations and citizens.

WHAT IT REQUESTS

Institutional support for a supervised pilot aligned with UHC, the National Digital Health Strategic Plan 2026-2030 and social protection.

POSITIONING · THREE STRATEGIC FUNCTIONS

FUNCTION 1

Mutual management

Modernise community health mutuals and strengthen their financial and administrative transparency: members, contributions, benefits, claims and SYSCOHADA accounting.

FUNCTION 2

Facilitating UHC

Mobilise health mutuals and community organisations to inform, guide and support people towards official enrolment, without ever replacing it.

FUNCTION 3

Coordinating health protection

Support complementarity between UHC, health mutuals and health facilities, avoiding double billing and reducing households' out-of-pocket costs.

Institutional safeguard. Function 2, and access to any UHC-related information, require an agreement with the competent administrations. Health data and personal data are processed in accordance with Law No. 2024/017 of 23 December 2024.

I · 1.1 — GENERAL CONTEXT

A young and increasingly connected population

30.1 M
estimated inhabitants, end of 2025

41.9%
of the population uses the Internet (12.6 million)

5.9 M
social media users

< 35
the age of the vast majority of Cameroonians

The National Development Strategy 2020-2030 (NDS30) puts human capital at the heart of its second pillar: equitable access to care, social protection and the roll-out of Universal Health Coverage by 2030.

I · 1.2 — HEALTH AND UNIVERSAL HEALTH COVERAGE

Households still pay for most of their care

72.2%
of current health expenditure paid directly by households

< 5%
of the population covered by insurance or a mutual (estimates)

4.6 M
people enrolled in UHC, 5.8 M pre-enrolled (April 2026)

35%
share of direct payments targeted for 2030 by the UHC strategy

UHC phase 1 was launched on 12 April 2023: children aged 0 to 5, pregnant women, HIV, tuberculosis and haemodialysis. There are 1.68 doctors per 10,000 inhabitants, and health accounts for 5.5% of the State budget. UHC currently operates by regulation; a UHC law is being prepared.

Sources: DataReportal, Digital 2026; Afrobarometer; NDS30; WHO, national health accounts; OECD; MINSANTE (29/04/2026); UHC strategy; NDS30 mid-term review.

I · 1.3 — PROBLEMS IDENTIFIED

Six obstacles to covering the risk of illness

01

Mutuals run on paper

Manual registers, no consolidated data, management that is hard to control.

02

Rights that cannot be checked at the desk

The health facility cannot tell whether the patient is up to date: fraud and refused coverage.

03

A poorly protected informal sector

Most workers have neither social security nor health coverage.

04

Data in silos

Digital health maturity rated 1.8 out of 5 in 2025; tools with little interoperability.

05

UHC still poorly known

UHC conditions, procedures and benefits remain poorly understood at community level.

06

Trust to be rebuilt

Financial management that is hard to trace discourages membership and regular contributions.

I · 1.4 — DIGITAL HEALTH

The State has set the course: an integrated, interoperable and sovereign ecosystem

2020 – 2024

PSNSN 1

Adopted on 16 January 2020, with 11.3 billion CFA francs. Strengthening of the health information system (DHIS2).

2026 – 2030

PSNSN 2

Launched on 4 March 2026, about 29 billion CFA francs, eight priority areas. Starting point: maturity of 1.8 out of 5.

VISION 2030

A single ecosystem

Integrated, interoperable, secure, inclusive and patient-centred, with sovereignty over health data.

Community health mutuals remain the least digitised link in this chain. This is where MX-CONNECT comes in, while opening itself to national standards.

PSNSN: National Digital Health Strategic Plan. Sources: MINSANTE, PSNSN 2020-2024 and 2026-2030; National Social Protection Policy 2017; ILO; Health Data Collaborative.

I · 1.5 — SOCIAL PROTECTION

Extending social protection beyond formal employment

11%

of the population benefits from at least one social protection programme (excluding health)

≈ 23%

social coverage at the NDS30 mid-term point

Efforts under way

- **National Social Protection Policy** (December 2017), which notably aims at a scheme suited to the informal sector.
- **Voluntary insurance of the National Social Insurance Fund (CNPS)** (2014 decree), open to self-employed and informal workers, at 8.4% of declared income.
- **Social economy**: Law No. 2019/004 recognises the mutual as a legal entity for welfare and solidarity.
- **Point of attention**: no text specific to health mutuals has been identified; a text on social security mutuals is mentioned in the UHC strategy.

I · 1.6 — YOUTH AND CIVIC ENGAGEMENT

Young people ready to commit, to be guided towards useful engagement

36%

of 18-35 year olds say they are unemployed and actively looking for work (Afrobarometer, 2024)

The framework set by MINJEC

- National Youth Policy (2006).
- National Volunteering Strategy (2015, with UNDP) and National Volunteering Programme.
- National Civic Service Agency for Participation in Development (ASCNPD).
- National Youth Council (2009) and Special Youth Three-Year Plan.

Skills, time and a strong presence on mobile phones: considerable potential for UHC awareness and volunteering.

Sources: OECD, Social protection financing in Cameroon; NDS30 review; National Social Protection Policy 2017; CNPS; Law No. 2019/004; Afrobarometer 2024; MINJEC; UNDP.

I · 1.7 — ENVIRONMENT AND LOCAL AREAS

The law calls for citizen participation; the tools are missing

Framework Law No. 96/12

Of 5 August 1996: principles of participation, information, prevention and subsidiarity.

Decentralisation

The General Code of Regional and Local Authorities (2019) gives councils a role.

Health and environment

Water, sanitation, hygiene: prevention relies on local action, often led by associations.

I · 1.8 — REFERENCE LAWS AND TEXTS

The framework MX-CONNECT operates in

AREA	REFERENCE TEXTS AND STRATEGIES
Health and UHC	Framework Law No. 96/03 of 4 January 1996 on health; Health Sector Strategy 2016-2027; UHC launched by regulation in 2023; UHC law being prepared.
Digital health and data	PSNSN 2026-2030; Law No. 2010/012 of 21 December 2010 on cybersecurity and cybercrime; Law No. 2024/017 of 23 December 2024 on personal data protection, in force since 23 June 2026 (creating the data protection authority, APDP).
Social protection and mutuals	National Social Protection Policy (2017); CNPS voluntary insurance (2014); Law No. 2019/004 of 25 April 2019 governing the social economy; Law No. 90/053 of 19 December 1990 on freedom of association.
Youth	National Youth Policy (2006); National Volunteering Strategy (2015); ASCNPD; National Youth Council (2009).
Environment and local areas	Framework Law No. 96/12 of 5 August 1996; Law No. 2019/024 of 24 December 2019 on the General Code of Regional and Local Authorities.
Development	National Development Strategy 2020-2030 (NDS30), pillar 2 “human capital and well-being”.

Numbers, dates and official English titles to be confirmed in the Official Gazette before circulation.

I · 1.9 — OPPORTUNITIES

The right moment to act

UHC has been launched

Millions of people are already enrolled: local relays are needed to inform, guide and support the others.

A new digital plan

The PSNSN 2026-2030 is starting and calls for interoperable, sovereign tools.

Clear rules on data

Law No. 2024/017, in force since June 2026, sets a framework of trust.

Mobile everywhere

12.6 million Internet users and widespread use of mobile phones and Mobile Money.

A tradition of solidarity

Tontines, associations, mutuals: existing networks to equip, not to create.

Young people ready to get involved

Volunteering and civic service can carry awareness and guidance.

OUR PROPOSAL

The missing link between public policies and communities

A single tool to modernise health mutuals, guide people towards UHC, check mutual entitlements at the desk and make visible the initiatives that improve people's lives.

II · 2.1 — OVERVIEW

One platform, three complementary components

COMPONENT 1 · AVAILABLE

Mutual management

- Members, households, dependants
- Contributions, receipts, treasury
- Benefits, claims, third-party payment
- Mutual membership card with QR code
- SYSCOHADA accounting, reports
- Health facility portal

COMPONENT 2 · TO BE DEVELOPED

Guidance towards UHC

- Information on UHC
- Voluntary identification of non-enrolled people
- Referral to official enrolment points
- Follow-up of referred people
- UHC status kept separate from mutual status
- Under agreement with MINSANTE

COMPONENT 3 · AVAILABLE

Initiatives network

- Pages of mutuals and associations
- Projects with verified results
- "Get involved", "Support"
- Offers, volunteering, jobs
- Online membership of a mutual
- Android app, invitations

II · 2.2 — EIGHT NON-NEGOTIABLE PRINCIPLES

PRINCIPLE	HOW THE PLATFORM APPLIES IT
Separated data	Each mutual has its own space; none can see another's data.
Medical confidentiality	No medical data in the social network or in notifications.
No funds held	The platform neither collects nor transfers money: it records transactions.
Genuine consent	Versioned documents, boxes never pre-ticked, time-stamped acceptances.
Two-factor authentication	One-time code mandatory for all staff.
Four-eyes control	No one validates their own entry; separate and incompatible roles.
SYSCOHADA accounts	Entries, trial balance, balance sheet and audit log available.
Honest figures	Real data only; declared information kept distinct from verified information.

Platform online: m.araild.com · Android app in beta. Registration with the data protection authority (APDP) is still to be done as part of the pilot.

II · 2.3 — CONTRIBUTION TO UNIVERSAL HEALTH COVERAGE

Health mutuals as community partners of UHC

Health mutuals are not UHC enrolment bodies. They are partners for mobilisation, information, guidance and support. The table below separates what a mutual does with MX-CONNECT from what remains the sole responsibility of the national scheme.

UHC OBJECTIVE	WHAT THE MUTUAL DOES WITH MX-CONNECT	WHAT THE NATIONAL SCHEME DOES
Make UHC known	Documented information for members and their families	Official content, eligibility conditions
Enrol	Voluntary identification of non-enrolled people, referral, help with files, follow-up	Enrolment, proof of enrolment, activation of entitlements
Reduce direct payments	Complementary benefits, mutual third-party payment	UHC benefit package and pricing
Secure coverage decisions	Logged check of mutual entitlements at the desk	Check of UHC entitlements
Avoid duplication	Coordination rules, checked invoices, four-eyes control	National coverage rules
Manage with data	Aggregated, anonymised indicators, once validated	National information system
Be accountable	SYSCOHADA accounting, activity reports	Supervision and regulation

Key point. MX-CONNECT does not replace the national UHC platform, issues no proof of enrolment and does not turn a mutual member into a UHC beneficiary. Any data exchange with national systems will follow the framework set by MINSANTE.

UHC strategy objective: reduce direct payments from 70% to 35% by 2030.

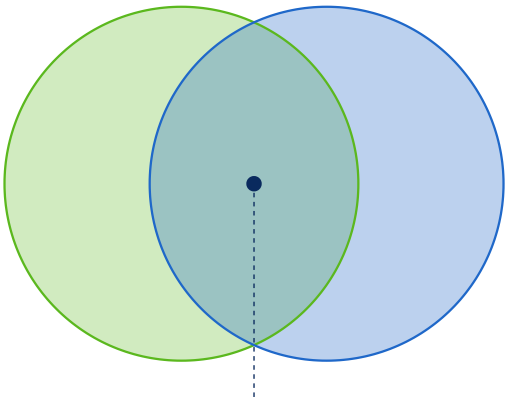
III · HEALTH MUTUALS AND UNIVERSAL HEALTH COVERAGE

Two distinct and complementary mechanisms

HEALTH MUTUAL

A voluntary act of solidarity

An individual or household receives the benefits defined by their mutual, under its statutes, its internal rules and the applicable regulations.



UNIVERSAL HEALTH COVERAGE

The State's national scheme

Conditions, procedures and eligibility criteria set by the competent authorities; enrolment follows official procedures.

SHARED AREA: COORDINATION AND COMPLEMENTARITY

Being a member of a health mutual does not mean being enrolled in UHC.

Enrolment in UHC does not make anyone a member of a mutual.

III · 3.1 — COORDINATION BETWEEN THREE ACTORS

A

Community health mutuals

They protect their members and become local relays.

B

The national UHC scheme

It enrols people, grants entitlements and sets coverage rules.

C

Health facilities and community structures

They provide care, check entitlements and apply the rules of each scheme.

MX-CONNECT intends to preserve this distinction while facilitating complementarity between the two mechanisms.

III · 3.2 — INDEPENDENT REGISTRATION PATHWAYS

Three pathways, never confused

PATHWAY 1

Joining a health mutual

Free and voluntary. The mutual sets its own conditions, contributions, benefits and coverage rules.

The mutual member file in MX-CONNECT

- Member's identity
- Household composition
- Dependants
- Membership date
- Contributions paid
- Benefits subscribed
- Mutual coverage status

Membership does not automatically give access to UHC.

PATHWAY 2

Enrolment in UHC

A separate procedure, following the official procedures of the competent authorities.

What MX-CONNECT can facilitate

- Informing people and raising awareness about UHC.
- Voluntary identification of people not yet enrolled.
- Referral to official enrolment points and help in putting files together.
- Follow-up of referred people.
- Checking enrolment status, once access to the official system is authorised.
- Identifying entitlements not yet activated or needing an update, under official procedures.

Limit. MX-CONNECT does not replace the national enrolment system and issues no proof of UHC enrolment. The platform facilitates guidance and follow-up, subject to a partnership and an authorised interconnection.

PATHWAY 3

Both forms of coverage together

This is where MX-CONNECT adds value: each person keeps two separate statuses, and the platform helps coordinate coverage according to the rules of each scheme. The four possible situations are shown on the next page.

III · 3.3 — SITUATION MATRIX

Every citizen has a place

Two independent questions are enough to place a person. In each situation, MX-CONNECT has a precise role, without ever confusing the two forms of coverage.

MEMBER OF A MUTUAL	ENROLLED IN UHC: YES	ENROLLED IN UHC: NO
YES	**3** **Member of a mutual and enrolled in UHC** Both forms of coverage managed separately; coverage coordinated under the applicable rules, without double billing.	**1** **Member of a mutual only** Management of their mutual coverage and a voluntary offer of guidance towards UHC.
NO	**2** **Enrolled in UHC only** Follow-up of their UHC status, under agreement, and information on complementary protection options.	**4** **Neither a member of a mutual nor enrolled in UHC** Awareness-raising, guidance towards UHC and information on available mutuals.

Inclusion. People who do not wish to join a mutual are not excluded: they can benefit from UHC.

Rigour. Members of a mutual are never considered automatically covered by UHC.

III · 3.4 — HOW HEALTH MUTUALS CAN ACCELERATE UHC

Five community relay functions

01 **Information relays**

Close to households, mutuals inform people about the conditions for accessing UHC, enrolment procedures and the benefits of coverage.

02 **Referral points for enrolment**

When a person joins, the manager offers to check their UHC status separately; if they are not enrolled, they are referred to the official scheme. This is voluntary and never a condition of membership.

03 **Relays to vulnerable populations**

Identifying people who struggle to access care — low-income households, informal workers, people far from health facilities — and referring them to the public mechanisms they are eligible for.

04 **Financial complementarity**

Depending on their means and rules, mutuals can offer complementary benefits:

- services not covered by UHC;
- costs left to the patient, where permitted;
- prevention and support;
- health assistance and community solidarity.

Coordination rules prevent double billing and double reimbursement of the same service.

05 **Community data for planning**

Aggregated data, once MINSANTE has validated the indicators and transmission arrangements:

- mutual members; people declaring they are enrolled;
- people not enrolled; people referred;
- enrolments actually confirmed;
- difficulties in accessing health coverage.

III · 3.5 — TRACKING THE TWO STATUSES SEPARATELY

Two files, two sources of truth

A · MUTUAL STATUS

Managed by the mutual

- Membership number
- Mutual the person belongs to
- Dependants
- Contributions
- Benefits
- Mutual coverage status

B · UHC STATUS · SEPARATE SPACE

Recorded under agreement

NOT ENROLLED

ENROLMENT IN PROGRESS

ENROLMENT CONFIRMED

TO BE UPDATED OR CHECKED

Only the official scheme, or an authorised check, can confirm an enrolment.

Declared ≠ confirmed. Unverified information ("the person says they are enrolled") is always displayed and counted separately from officially confirmed data.

III · 3.6 — COMPLEMENTARY COVERAGE

Strengthening financial protection without double billing

When a person benefits from both mechanisms, coverage arrangements are set by the agreements, the mutual's benefits and the national UHC rules. Complementarity may cover:

i. complementary services;

ii. any costs left to the patient;

iii. services covered by neither mechanism, within authorised limits.

Objective: strengthen households' financial protection, avoid double billing and reduce direct health spending.

The "UHC status" module and the coordination rules are functions to be developed during the pilot, with the competent services.

III · 3.7 — A MEASURABLE CONTRIBUTION TO EXTENDING UHC

What MX-CONNECT aims to improve

Knowledge — increase knowledge of UHC at community level.

Barriers — reduce administrative and information barriers.

Data — produce aggregated data on needs and access difficulties.

Guidance — facilitate access to official enrolment mechanisms.

Follow-up — better track referred people until confirmation.

Complementarity — strengthen the link between public protection and mutual solidarity.

INSTITUTIONAL POSITIONING

MX-CONNECT does not create a parallel UHC scheme and does not turn members of health mutuals into automatic UHC beneficiaries.

The platform is a digital tool supporting community mobilisation, mutual management, guidance towards the national scheme and the follow-up of complementarity between health protection mechanisms.

Condition for implementation. Access to UHC information, verification of enrolments and any interconnection of systems require an agreement with the competent administrations. Health data and personal data are processed in accordance with the regulations in force.

IV · 4.1 — DIGITAL HEALTH (PSNSN 2026-2030)

Digitising the community link, while opening up to the national ecosystem

AVAILABLE TODAY

- Digital file of the member and their dependants
- Mutual card with a signed QR code, read by camera or photo
- Portal for contracted health facilities
- Checked PDF invoices, CSV exports
- Android app and mobile website

TO BE BUILT WITH MINSANTE

- Separate tracking of mutual and UHC statuses
- Secure exchange with the national UHC platform
- Aggregated, anonymised indicators to DHIS2
- National register of health facilities
- PSNSN interoperability and hosting standards

IV · 4.2 — SOCIAL PROTECTION, YOUTH, ENVIRONMENT

The social network serving three public policies

MINTSS

Social protection

- Mutuals open to the informal sector
- Traceable contributions: schedules, receipts, statements
- Transparent management members can check
- Information bridge to CNPS voluntary insurance (to be studied)

MINJEC

Youth

- "Get involved": volunteering, skills, relays
- Volunteering, internship and job offers
- Young ambassadors for UHC and mutuals
- Inviting friends and family via WhatsApp

LOCAL AREAS

Environment

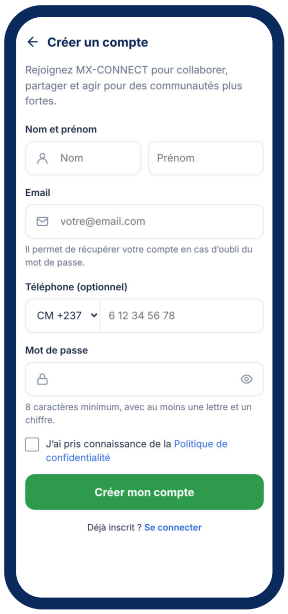
- "Environment" among the areas of action
- Projects with declared, then verified results
- Mobilising volunteers and partners
- Visibility of local action for councils

V · 5.1 — THE CITIZEN

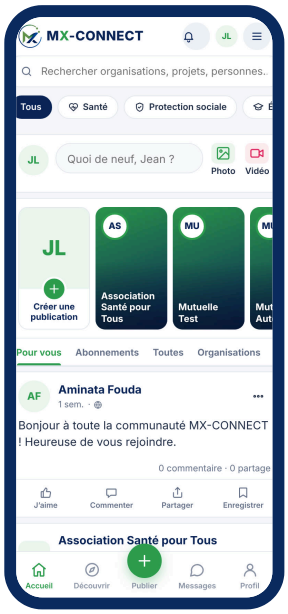
Open the app, create an account, follow the news



Start · version check and routing



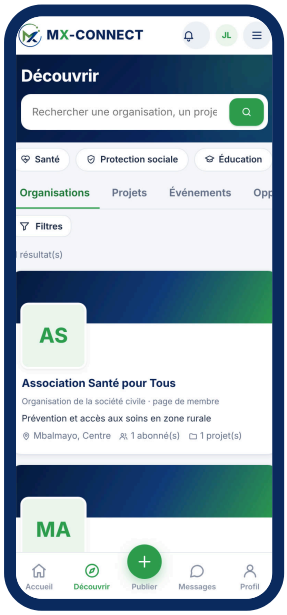
Sign-up · e-mail or phone, explicit consent



Home · news, search, areas of action

V · 5.2 — INITIATIVES

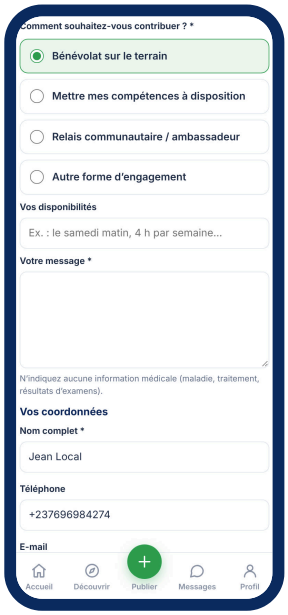
Discover a project, see its results, get involved



Discover · organisations, projects, filters



Results · declared, then verified against evidence

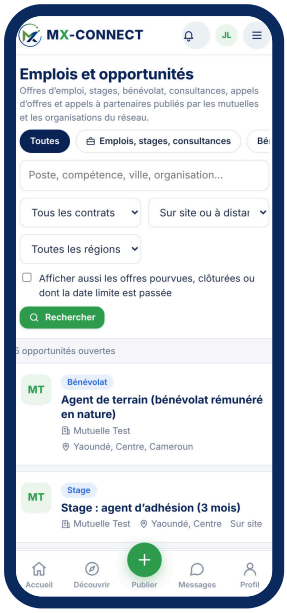


Get involved · volunteering, skills, relays

Screenshots taken on a test copy of the platform with fictitious data. The interface is currently in French.

V · 5.3 — YOUTH AND MOBILISATION

Find an opportunity, manage your space, invite friends and family



Opportunities · volunteering, internships, jobs



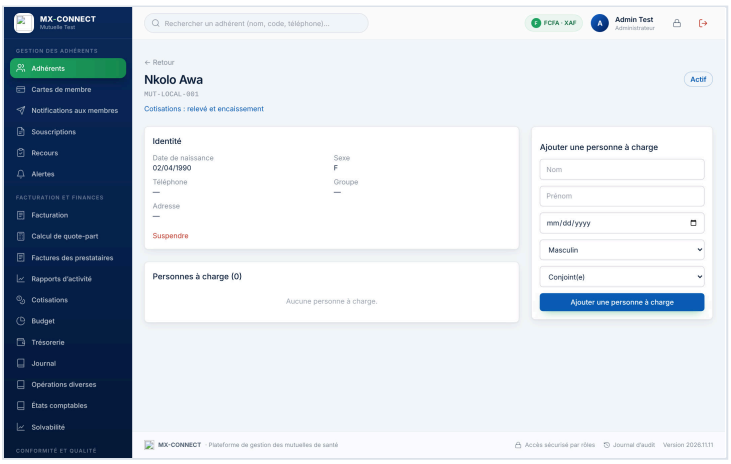
My profile · posts, groups, requests



Invite a friend · WhatsApp, Facebook, SMS

V · 5.4 — THE MUTUAL: MEMBERS AND MEMBERSHIP CARD

Register members, issue a verifiable mutual membership card

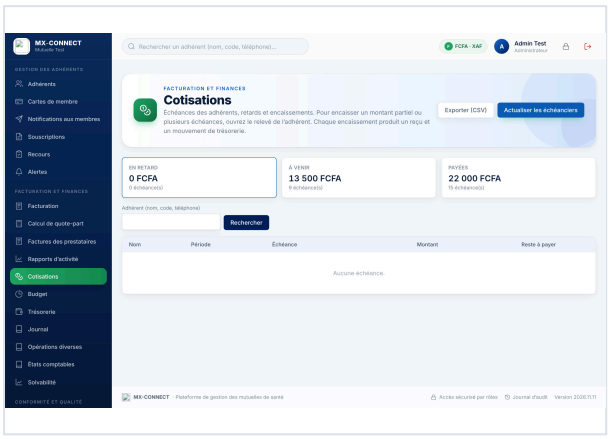




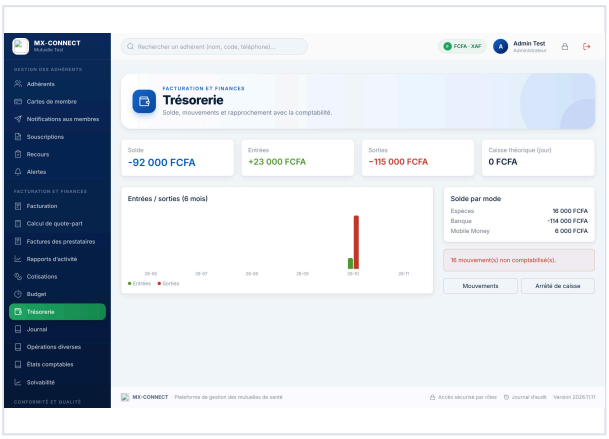
A mutual card, not a UHC card. It only certifies membership of the mutual and its benefits; it proves no UHC enrolment.

V · 5.5 — CONTRIBUTIONS AND TREASURY

Every franc collected is traced, with a receipt



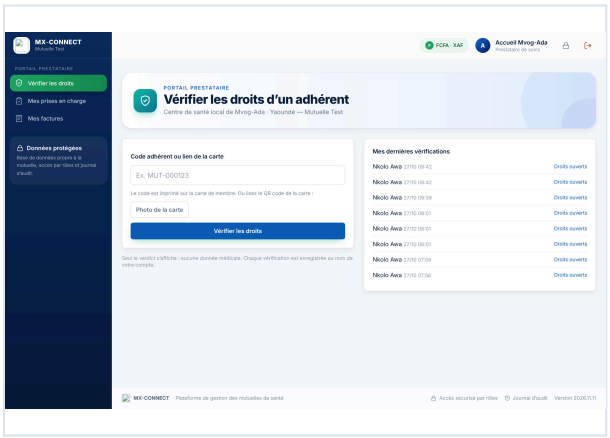
Contributions · schedules, arrears, part payments, single receipt



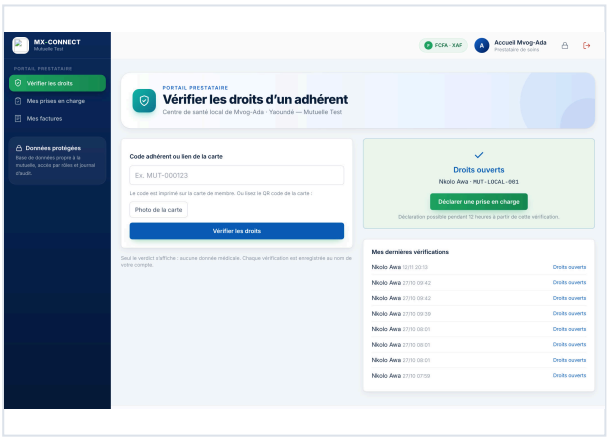
Treasury · cash, bank, Mobile Money, daily cash closing

V · 5.6 — THE HEALTH FACILITY

Check mutual entitlements in a few seconds



1. Card read by camera or photo, or code typed in

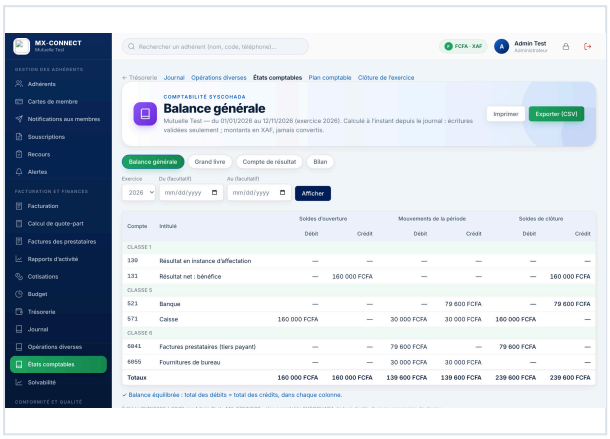


2. "Entitlements open" answer, then the claim is declared

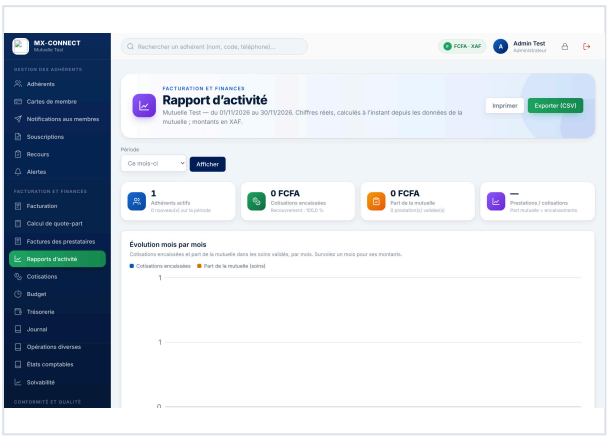
Mutual entitlements ≠ UHC entitlements. Today the portal checks mutual entitlements only. Checking UHC entitlements is the national scheme's responsibility; coordination between the two will be defined with MINSANTE.

V · 5.7 — ACCOUNTABILITY

SYSCOHADA accounts and reports ready to submit



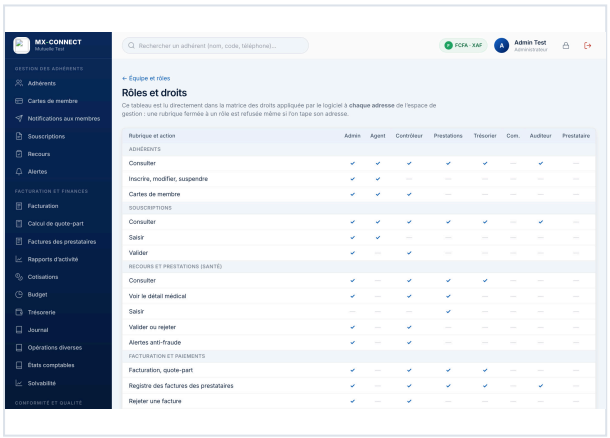
Financial statements · trial balance, ledger, income statement, balance sheet, year-end closing



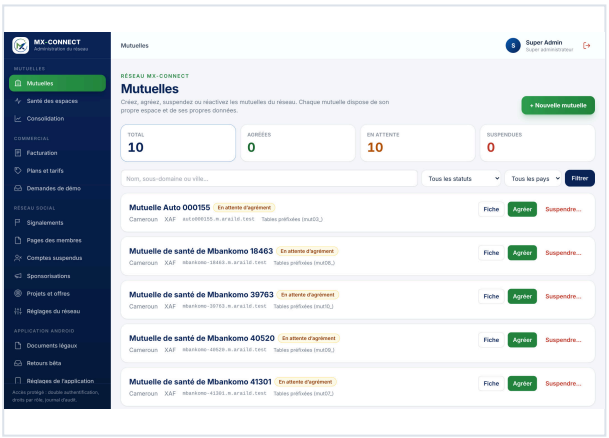
Activity reports · members, contributions, claims, trends

V · 5.8 — GOVERNANCE

Separate roles in each mutual, oversight of the network



Roles and permissions · officer, controller, treasurer, auditor, provider



Network space · creation, approval and suspension of mutuals

Screenshots taken on a test copy of the platform with fictitious data. The interface is currently in French.

VI · 6.1 — WHAT WE ARE REQUESTING

A framework, a pilot and technical endorsement

No purchase request. We propose a partnership agreement and a joint technical committee, with one focal point per ministry, ARAILD and representatives of the mutuals.

MINSANTE

Public Health

- Recognition of MX-CONNECT as a tool for mutual management and community guidance towards UHC.
- An agreement specifying access to UHC information, verification of enrolments and interconnection.
- Access to PSNSN interoperability standards and validation of the indicators.
- A pilot in one health district, with its health facilities.

MINTSS

Labour and Social Security

- Support in structuring informal-sector mutuals.
- Study of an information bridge with CNPS voluntary insurance.
- Contribution to discussions on the legal framework for mutuals.

MINJEC

Youth Affairs and Civic Education

- Outreach through the National Youth Council and associations.
- “Get involved” as a volunteering channel, with ASCNPD.
- Training young ambassadors for UHC and mutuals.

The environment ministry (MINEPDED) and councils could be involved at a later stage.

VI · 6.2 — PROPOSED ROADMAP

An 18-month pilot, measured and evaluated

MONTHS 0 – 3 · SET-UP

- Agreement and technical committee
- Choice of the district and of [__] mutuals
- Registration with the APDP
- Specification of the "UHC status" module

MONTHS 3 – 12 · PILOT

- Training of managers
- Memberships and mutual cards
- UHC awareness and guidance
- Entitlement checks in [__] health facilities

MONTHS 12 – 18 · EVALUATION

- Joint evaluation
- Interoperability with UHC
- Decision on scale-up

VI · 6.3 — PILOT INDICATORS

INDICATOR	DEFINITION
Registered members · collection rate	Mutual management: active members and contributions collected / due.
Entitlement checks · traced claims	Operations logged at health facility desks.
Members made aware of UHC	People who received documented information on UHC.
People identified as not enrolled	Non-enrolment established or confirmed under authorised arrangements.
People referred for enrolment	People who received a documented referral.
Confirmed UHC enrolments	Confirmation from the official scheme or an authorised check.
Enrolment confirmation rate	Confirmed enrolments / people referred × 100.
People covered by both mechanisms	Mutual membership and UHC enrolment both confirmed.
Young people involved · projects with verified results	Citizen component and community mobilisation.

Counting rule. A person who has only been referred is never counted as enrolled. The [__] targets are to be set with the technical committee.

APPENDIX — SOURCES

Sources consulted

DATA	SOURCE
Population, Internet, social media	DataReportal, *Digital 2026: Cameroon*
Youth, employment of 18-35 year olds	Afrobarometer, dispatch AD1222 (2026), 2024 survey
Share of direct payments (72.2%)	WHO, national health accounts; L'Économie; OECD (≈ 70% in 2021)
Insurance and mutual coverage	Press and study estimates (1 to 5%); ILO, social protection assessment
UHC: launch, enrolment	MINSANTE; WHO AFRO; Minister of Public Health's update of 29 April 2026; UHC development strategy
Digital health	MINSANTE, PSNSN 2020-2024 and PSNSN 2026-2030; Health Data Collaborative
Social protection	OECD, *Social protection financing in Cameroon*; National Social Protection Policy (2017); CNPS
NDS30, mid-term review	Journal du Cameroun; NDS30 documents
Personal data	Law No. 2024/017 of 23 December 2024; specialist press on its entry into force
Social economy	Law No. 2019/004 of 25 April 2019
Youth and volunteering	MINJEC, National Youth Policy; UNDP, National Volunteering Strategy
Environment	Law No. 96/12 of 5 August 1996 (FAOLEX); Law No. 2019/024

Disclaimer. Figures are quoted as published by their sources, which do not always measure the same thing. Legal references, and their official English titles, should be confirmed in the Official Gazette before any official circulation.

MX-CONNECT

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IN CONCLUSION

Equipping communities so that health protection reaches every household.

- The health mutual protects its members according to its own benefits.
- UHC provides coverage within the national scheme.
- MX-CONNECT facilitates coordination and complementarity between the two.

MX-CONNECT is ready to be put to the test, under the authority of the ministries, within a transparent, contractual and evaluated framework.

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